

Hypospadias

» What is this?

This condition occurs in about one in 300 baby boys when the urethra exits the penis along the shaft, rather than at the tip of the penis. Depending upon where the hypospadias occurs, the penis may not be able to straighten properly, and the foreskin may not develop normally on the underside.

» What can be done?

If the urethra appears close to the tip of the penis, the condition is considered mild and often it is left as it is. However, in more severe forms of the condition, which can cause pain when urinating and later difficulties with erectile function, are usually treated with surgery before the baby reaches 18 months old.

Pyloric stenosis

» What is this?

Around one in 500 babies are born with this condition. The pylorus is a muscular tube that connects the stomach to the bowel. Sometimes a baby may be born with a section of overdeveloped pylorus muscle and this means that milk can't pass beyond the stomach. This causes the baby to vomit back up her feedings (sometimes violently) and in turn she becomes dehydrated and undernourished. It can sometimes be a hereditary condition, and tends to affect more boys than girls. It usually develops at about six weeks after birth.

» What can be done?

Your baby will need to have surgery (often keyhole surgery) under general anesthesia to cut through the muscle and create a passage for milk to pass into the bowel from the stomach. There tend to be no long-term side effects, and babies make a full recovery, going on to feed perfectly normally throughout life.

Imperforate anus

» What is this?

This is a rare condition affecting about just one in every 5,000 babies. In the uterus, the entirety of your baby's digestive system begins, essentially, as one long tube, separated by thin membranes that gradually break down as necessary as the different sections of the gastrointestinal tract form. Sometimes, the sections that form the large intestine, anus, and rectum don't form properly, making it impossible for your baby to pass a stool. This is an imperforate anus.

» What can be done?

Imperforate anus is corrected using surgery, usually as soon as the problem is discovered (or at birth—since often it shows up on an ultrasound). Your baby will need to wear

a colostomy bag until the bowel has healed. Most children go on to have a perfectly healthy and well-functioning bowel, but occasionally a child may need to go on to a special diet to relieve constipation.

Fetal alcohol syndrome

» What is this?

Fetal alcohol syndrome (FAS) occurs when the levels of alcohol a mother drinks during pregnancy affect the development of the baby. A newborn with FAS often has a small head with distinctive facial features, including

droopy eyelids with skin folds in the upper inner corners, and small eyes. Ears may be set low down and the mouth may have an underdeveloped jaw, a thin upper lip, and only a shallow or indistinct groove above the lip (called the philtrum). The bridge of the baby's nose will seem low when compared with the position of the eyes, and the nose itself may seem short and upturned. In addition, FAS can cause heart problems and learning difficulties. It is estimated that about one in 1,000 children are affected by FAS.

» What can be done?

There is no treatment for FAS itself, although any heart abnormalities may be operable, and your baby will need support throughout childhood for any learning or behavioral difficulties.

Cerebral palsy

» What is this?

Cerebral palsy is the result of lack of oxygen to the brain, which can occur before a baby is born (causing damage directly to the neurons in the brain as the brain forms), during birth (as a result of hemorrhage), or as a consequence of an illness such as meningitis after birth. It affects areas of the brain that control muscle movement (including those for speech and swallowing), hearing, vision, and learning.

» What can be done?

There is no cure for the condition, but it is not progressive—that is, it doesn't get worse over time. This means that management can be uniquely tailored to each individual child. Once your child has been diagnosed with cerebral palsy, you will have a team of health professionals involved in her care. You will also have one key worker, who may be your point of contact and will help to coordinate the management of your child's condition. There is a wide range of therapies for cerebral palsy that help deal with different aspects of the condition, with the goal of relieving symptoms and maximizing your child's independence. Physical therapy is one of the main treatments, which is important to prevent muscles from weakening further and movement from becoming more restricted. Occupational therapy may be given to help your child manage everyday



Massage therapy Complementary therapies such as massage gently manipulate soft body tissues, helping to relax and relieve muscle stiffness.

activities and tasks. Speech therapy provides exercises to help your child communicate more clearly if needed. Speech and language therapists can also help children reduce drooling. If the muscles are very stiff, your child may need medication to relax them. Occasionally surgery is needed to treat bone and joint problems.

Cerebral palsy occurs in about 3 in 1,000 births in the US, and it is more common in boys than girls. More than half the cases each year occur as a result of premature births.